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ESOMAC

Esomeprazole is a CYP2C19 inhibitor. When starting or ending treatment with esomeprazole, the potential for interactions with drugs metabolised through CYP2C19 should be considered. An interaction is observed between clopidogrel and esomeprazole (see Drug interactions). The clinical relevance of this interaction is uncertain. As a precaution, concomitant use of esomeprazole and clopidogrel should be discouraged.

When prescribing esomeprazole for on demand therapy, the implications for interactions with other pharmaceuticals, due to fluctuating plasma concentrations of esomeprazole should be considered. (see Drug interactions).

Interference with laboratory tests

Increased Chromogranin A (CgA) level may interfere with investigations for neuroendocrine tumours. To avoid this interference, esomeprazole treatment should be stopped for at least 5 days before CgA measurements (see Pharmacodynamics). If CgA and gastrin levels have not returned to reference range after initial measurement, measurements should be repeated 14 days after cessation of proton pump inhibitor treatment.

Excipients:

This medicinal product contains sucrose. Patients with rare hereditary problems of fructose intolerance, glucose-galactose malabsorption or sucrase-isomaltase insufficiency should not take this medicine.

Interactions with Other Medicaments

Effects of esomeprazole on the pharmacokinetics of other drugs

Protease inhibitors

Omeprazole has been reported to interact with some protease inhibitors. The clinical importance and the mechanisms behind these reported interactions are not always known. Increased gastric pH during omeprazole treatment may change the absorption of the protease inhibitors. Other possible interaction mechanisms are via inhibition of CYP2C19.

For atazanavir and nelfinavir, decreased serum levels when given together with omeprazole and concomitant administration is not recommended. Co-administration of omeprazole (40 mg once daily) with atazanavir 300 mg/ritonavir 100 mg resulted in a substantial reduction in atazanavir exposure. Increasing the atazanavir dose to 400 mg did not compensate for the impact of omeprazole on atazanavir exposure. The co-administration of omeprazole (20 mg qd) with atazanavir 400 mg/ritonavir 100 mg resulted in a decrease in the atazanavir exposure. Co-administration of omeprazole (40 mg qd) reduced mean nelfinavir AUC, C_{max} and C_{min} and mean AUC, C_{max} and C_{min} for the pharmacologically active metabolite M8. Due to the similar pharmacodynamic effects and pharmacokinetic properties of omeprazole and esomeprazole, concomitant administration with esomeprazole and atazanavir is not recommended (see Warning and precautions) and concomitant administration with esomeprazole and nelfinavir is contraindicated (see Contraindications).

For saquinavir (with concomitant ritonavir), increased serum levels have been reported during concomitant omeprazole treatment (40 mg qd). Treatment with omeprazole 20 mg qd had no effect on the exposure of darunavir (with concomitant ritonavir) and amprenavir (with concomitant ritonavir). Treatment with esomeprazole 20 mg qd had no effect on the exposure of lopinavir (with concomitant ritonavir).

Methotrexate

When given together with PPIs, methotrexate levels have been increased. In high-dose methotrexate administration a temporary withdrawal of esomeprazole may need to be considered.

Tacrolimus

Concomitant administration of esomeprazole has been reported to increase the serum levels of tacrolimus. A reinforced monitoring of tacrolimus concentrations as well as renal function (creatinine clearance) should be performed, and dosage of tacrolimus adjusted if needed.

Medicinal products with pH dependent absorption

Gastric acid suppression during treatment with esomeprazole and other PPIs might decrease or increase the absorption of medicinal products with a gastric pH dependent absorption. As with other medicinal products that decrease intragastric acidity, the absorption of medicinal products such as ketoconazole, itraconazole and efotimib can decrease and the absorption of digoxin can increase during treatment with esomeprazole. Concomitant treatment with omeprazole (20 mg daily) and digoxin increased the bioavailability of digoxin. Digoxin toxicity has been rarely reported. However, caution should be exercised when esomeprazole is given at high doses in elderly patients. Therapeutic drug monitoring of digoxin should then be reinforced.

Medicinal products metabolised by CYP2C19

Esomeprazole inhibits CYP2C19, the major esomeprazole-metabolising enzyme. Thus, when esomeprazole is combined with drugs metabolised by CYP2C19, such as diazepam, citalopram, mirtapramine, domipramine, phenytoin etc., the plasma concentrations of these drugs may be increased and a dose reduction could be needed. This should be considered especially when prescribing esomeprazole for on-demand therapy.

Diazepam

Concomitant administration of 30 mg esomeprazole decreased clearance of the CYP2C19 substrate diazepam.

Phenytoin

Concomitant administration of 40 mg esomeprazole increased trough plasma levels of phenytoin in epileptic patients. It is recommended to monitor the plasma concentrations of phenytoin when treatment with esomeprazole is introduced or withdrawn.

Voriconazole

Omeprazole (40 mg once daily) increased voriconazole (a CYP2C19 substrate) C_{max} and AUC₀₋₁₂.

Cilostazol

Omeprazole as well as esomeprazole act as inhibitors of CYP2C19. Omeprazole, increased C_{max} and AUC for cilostazol.

Cisapride

Concomitant administration of 40 mg esomeprazole increased area under the plasma concentration-time curve (AUC) and prolongelimination half-life (t_{1/2}) but no significant increase in peak plasma levels of cisapride. The slightly prolonged QTc interval observed after administration of cisapride alone, was not further prolonged when cisapride was given in combination with esomeprazole (see Warning and precautions).

Warfarin

Concomitant administration of 40 mg esomeprazole to warfarin showed that coagulation times were within the accepted range. Monitoring is recommended when initiating and ending concomitant esomeprazole treatment during treatment with warfarin or other coumarine derivatives.

Clopidogrel

A pharmacokinetic (PK) pharmacodynamic (PD) interaction between clopidogrel (300 mg loading dose/75 mg daily maintenance dose) and esomeprazole (40 mg p.o.daily) decreased exposure to the active metabolite of clopidogrel and decreased maximum inhibition of (ADP induced) platelet aggregation.

Inconsistent data on the clinical implications of a PK/PD interaction of esomeprazole in terms of major cardiovascular events have been reported. As a precaution concomitant use of clopidogrel should be discouraged.

Investigated medicinal products with no clinically relevant interaction

Amoxicillin and quinidine

Esomeprazole has been shown to have no clinically relevant effects on the pharmacokinetics of amoxicillin or quinidine.

Naproxen or rofecoxib

Concomitant administration of esomeprazole and either naproxen or rofecoxib did not identify any clinically relevant pharmacokinetic interactions during short-term studies.

Effects of other medicinal products on the pharmacokinetics of esomeprazole

Medicinal products which inhibit CYP2C19 and/or CYP3A4

Esomeprazole is metabolised by CYP2C19 and CYP3A4. Concomitant administration of esomeprazole and a CYP3A4 or CYP2C19 inhibitor (such as clarithromycin or voriconazole) increased esomeprazole exposure. A dose adjustment of esomeprazole is not regularly required in either of these situations. However, dose adjustment should be considered in patients with severe hepatic impairment and if long-term treatment is indicated.

Medicinal products which induce CYP2C19 and/or CYP3A4

Drugs known to induce CYP2C19 or CYP3A4 or both (such as rifampicin and St. John's wort) may lead to decreased esomeprazole serum levels by increasing the esomeprazole metabolism.

Paediatric population

Interaction studies have only been performed in adults.

Fertility, pregnancy and lactation

Pregnancy

Clinical data on exposed pregnancies with esomeprazole are insufficient. Esomeprazole do not indicate direct or indirect harmful effects with respect to embryonal/fetal development, pregnancy, parturition or postnatal development. Caution should be exercised when prescribing to pregnant women.

Esomeprazole do not indicate direct or indirect harmful effects with respect to reproductive toxicity.

Lactation

It is not known whether esomeprazole is excreted in human breast milk. There is insufficient information on the effects of esomeprazole in newborns/infants. Esomeprazole should not be used during lactation.

Fertility

Esomeprazole with the racemic mixture omeprazole, given by oral administration do not indicate effects with respect to fertility.

Undesirable Effects

List of adverse reactions

System Organ Class	Frequency	Undesirable Effect
Blood and lymphatic system disorders	Rare	Leukopenia, thrombocytopenia
	Very rare	Agranulocytosis, pancytopenia
	Rare	Hypersensitivity reactions e.g. fever, angioedema and anaphylactic reaction/shock
Immune system disorders	Rare	Hypersensitivity reactions e.g. fever, angioedema and anaphylactic reaction/shock
Infections & infestations		Clostridium difficile associated diarrhoea.
Metabolism and nutrition disorders	Uncommon	Peripheral oedema
	Rare	Hyponatraemia
	Not known	Hypomagnesaemia (see Warning and precautions); severe hypomagnesaemia can correlate with hypocalcaemia. Hypomagnesaemia may also be associated with hypokalaemia.
		Vitamin B12 deficiency
Psychiatric disorders	Uncommon	Insomnia
	Rare	Agitation, confusion, depression
	Very rare	Aggression, hallucinations
Nervous system disorders	Common	Headache
	Uncommon	Dizziness, paraesthesia, somnolence
	Rare	Taste disturbance
Eye disorders	Rare	Blurred vision
Ear and labyrinth disorders	Uncommon	Vertigo
Respiratory, thoracic and mediastinal disorders	Rare	Bronchospasm
Gastrointestinal disorders	Common	Abdominal pain, constipation, diarrhoea, flatulence, nausea/vomiting, fundic gland polyps (benign)
	Uncommon	Dry mouth
	Rare	Stomatitis, gastrointestinal candidiasis
	Not known	Microscopic colitis
	Uncommon	Increased liver enzymes
Hepatobiliary disorders	Rare	Hepatitis with or without jaundice
	Very rare	Hepatic failure, encephalopathy in patients with pre-existing liver disease
	Uncommon	Dermatitis, pruritus, rash, urticaria
Skin and subcutaneous tissue disorders	Rare	Alopecia, photosensitivity
	Very rare	Erythema multiforme, Stevens-Johnson syndrome, toxic epidermal necrolysis (TEN)
	Not known	Subacute cutaneous lupus erythematosus (see Warning and precautions
	Uncommon	Fracture of the hip, wrist or spine (see Warning and precautions
Musculoskeletal and connective tissue disorders	Rare	Arthralgia, myalgia
	Very rare	Muscular weakness

Renal and urinary disorders	Very rare	Interstitial nephritis; in some patients renal failure has been reported concomitantly.
Reproductive system and breast disorders	Very rare	Gynaecomastia
General disorders and administration site conditions	Rare	Malaise, increased sweating

Reporting of suspected adverse reactions

Reporting suspected adverse reactions after authorisation of the medicinal product is important. It allows continued monitoring of the benefit/risk balance of the medicinal product. Healthcare professionals are asked to report any suspected adverse reactions via the local reporting system.

Symptoms and Treatment of Overdose

There is very limited experience to date with deliberate overdose. The symptoms described in connection with 280 mg were gastrointestinal symptoms and weakness. Single doses of 80 mg esomeprazole were uneventful. No specific antidote is known. Esomeprazole is extensively plasma protein bound and is therefore not readily dialyzable. As in any case of overdose, treatment should be symptomatic and general supportive measures should be utilized

Effects on ability to drive and use machine

Esomeprazole has minor influence on the ability to drive and use machines. Adverse reactions such as dizziness (uncommon) and blurred vision (rare) has been reported (see Side effects). If affected patients should not drive or use machines.

Instructions for Use

The tablets should be swallowed whole with liquid. The tablets should not be chewed or crushed. For patients who have difficulty in swallowing, the tablets can also be dispersed in half a glass of non-carbonated water. No other liquids should be used as the enteric coating may be dissolved. Stir until the tablets disintegrate and drink the liquid with the pellets immediately or within 30 minutes. Rinse the glass with half a glass of water and drink. The pellets must not be chewed or crushed.

For patients who cannot swallow, the tablets can be dispersed in non-carbonated water and administered through a gastric tube. It is important that the appropriateness of the selected syringe and tube is carefully tested.

Administration through gastric tube

1. Put the tablet into an appropriate syringe and fill the syringe with approximately 25 ml water and approximately 5 ml air. For some tubes, dispersion in 50 ml water is needed to prevent the pellets from clogging the tube.
2. Immediately shake the syringe for approximately 2 minutes to disperse the tablet.
3. Hold the syringe with the tip up and check that the tip has not clogged.
4. Attach the syringe to the tube whilst maintaining the above position.
5. Shake the syringe and position it with the tip pointing down. Immediately inject 5–10 ml into the tube. Invert the syringe after injection and shake (the syringe must be held with the tip pointing up to avoid clogging of the tip).
6. Turn the syringe with the tip down and immediately inject another 5–10 ml into the tube. Repeat this procedure until the syringe is empty.
7. Fill the syringe with 25 ml of water and 5 ml of air and repeat step 5 if necessary to wash down any sediment left in the syringe. For some tubes, 50 ml water is needed.

Incompatibility

Not applicable.

Shelf-Life

24 months

Storage and Handling Instruction

Store below 30°C

No special requirements for disposal.

Packaging Information

Carton containing 3 Alu-Alu blister of 10 Tablets each

Manufactured By

Cipla LTD.
L-139 to L-146
Verna Industrial Estate,
Verna Goa, India

Product Registration Holder

Cipla Malaysia Sdn Bhd
Suite 1101, Amcorp Tower,
Amcorp Trade Centre
18 Persiaran Barat, 46050,
Petaling Jaya, Selangor Malaysia

Last Updated: January 2020

Cipla

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ARTIST PATH	
MATERIAL	ITC paper with perforated tape printed with "Tear here"
NO. OF COLOUR	1
PANTONE	Black-Black
INK DETAILS	Oil based Ink from DIC OR MICRO
LENGTH	472 mm $\pm$ 2 mm
LENGTH AFTER FOLDING	59 mm $\pm$ 2 mm
WIDTH	180 mm $\pm$ 2 mm
WIDTH AFTER FOLDING	30 mm $\pm$ 2 mm
GRAMMAGE	NLT 35 gsm
MATERIAL SPECIFICATION	40 GSM ITC Paper with perforated tape printed with "↑Tear Here↑"
PHARMACODE DIRECTION	31869
TYPE	Booklet
DATE	03-October-2022
ARTWORK PRINT SIZE	Actual
SUPERSEDES/REFERENCE	Supersedes: 21085501
ENGVIEW KEYLINE	NA
INSTRUCTIONS/REMARK	- Any deviation must be brought to the notice of packaging development co-ordinator immediately. - For any clarification, please contact packaging development co-ordinator immediately. - NO CHANGES IN ARTWORK SHOULD BE DONE BY THE PRINTER - The printer should verify the e-proof against the approved artwork before submitting for approval and the e-proof should have printer details.

Finalization Summary			
ACTION	DEPARTMENT	ACTION BY	ACTION ON
Prepared by	Packaging Officer	Vinod Vishwakarma	23-Sep-2022 09:39:00
Uploaded by	Packaging Artist	Vaibhav Sawant	26-Sep-2022 11:47:00
Checked by	Packaging Officer	Vinod Vishwakarma	03-Oct-2022 15:08:00
Checked by	Packaging Approver	Maxi Rodricks	07-Oct-2022 21:33:00
Checked by	Regulatory Affairs	Kavita Gawade	13-Oct-2022 15:39:00
Checked by	Licensing	Monika Lakhani	27-Dec-2022 11:28:00
Approved by	Corporate Quality Assurance	Bina Mehta	28-Dec-2022 13:44:00